resources get reallocated into producing goods and services that people actually like, such as movies and clothes. When accidents are prevented, not only does society reduce expenditure on healthcare, we become better off because those same resources are shifted to producing things that people actually want.

A world with fewer road accidents is thus one where some individuals experience less pain and suffering, there is less lost output, and society gains happiness as some resources switch from producing healthcare services to producing things that people actually like. At Indian levels of road safety, this is better than producing healthcare services to address the problem of road accidents.

There is something awkward in counting the production of healthcare services, associated with preventable problems, as part of GDP. When air quality in India gets worse, there is greater purchase of healthcare services, and GDP as conventionally counted goes up. This detracts from the notion of higher GDP as synonymous with higher consumption and choice in the hands of the people. In a good country, we would spend *less* on healthcare as fewer people would get sick. We suspect that GDP will become a better measure of consumption and welfare if the healthcare expenditures, associated with preventable events, are *subtracted* from GDP.

The case for state intervention in health

The standard recipe of public economics is to start from conditions with no state, and identify the market failures. What goes wrong when we have pure laissez faire, when the government does nothing about health? There is a neat split between two classes of market failure.

Public health: Consider the eradication of smallpox in India in 1975. [1] Once smallpox was eliminated, everyone gained welfare: this joy is non-rival and non-excludable. Controlling communicable diseases, and controlling sources of ill health such as road safety or natural disasters—these are public goods.